



PROCEDURE

Transfer of a Consumer within Adult Community Services

Scope (Staff):	All NMHS Mental Health (MH) Adult Community Staff
Scope (Area):	NMHS MH Adult Community Areas

1. Aim

This procedure provides direction for all workers involved in arranging the transfer of a consumer on the background of an expectation and understanding that we work cooperatively and with collegiality to ensure the safe transfer of consumers.

2. Risk

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☐
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Definitions

Assessment and Treatment Team - ATT: The North Metropolitan Health Service (NMHS) Mental Health Assessment and Treatment Team (ATT) function as the first point of contact with community mental health services. Following triage and assessment, this team will facilitate the most appropriate type of care (crisis intervention, community or inpatient) for the consumer for up to 90 days.

The ATT provides triage and multidisciplinary mental health services to adult consumers with acute care needs in a community setting. These services are available after hours, weekends and on public holidays. ATT services are provided in the consumers home, a community mental health clinic or other nominated places.

Continuing Treatment Teams - CTT: Continuing Treatment Teams (CTT) receive referrals from the local Assessment and Treatment Team (ATT) for people who have severe and enduring mental illness and require ongoing treatment and support. The CTT's are a secondary point of contact in the community public mental health service clinical pathway.

The CTT offers biopsychosocial, evidence-based interventions and collaborative recovery goals and provides support to consumers in the home, the community clinic, or in other nominated locations as clinically appropriate. Regular review of the treatment and collaborative plans are an integral component of CTT service provision.

Active Recovery Team - ART: The North Metropolitan Health Service (NMHS) Mental Health Active Recovery Team (ART) model of care provides coordinated, responsive, and tailored wrap around mental health and psychosocial treatment and support for up to a 90-day period in partnership between Wanneroo and Stirling Community Mental Health Services and Luma and in collaboration with general practitioners and other community agencies.

Connect to Community - C2C: The North Metropolitan Health Service (NMHS) Mental Health Connect to Community (C2C) service provides mental health assessment and interventions for consumers who are transitioning from an inpatient admission to community mental health services who reside in the Lower West and Stirling catchment areas and have not previously been active with community mental health services. The C2C is a multidisciplinary service. Referrals are accepted from Graylands Hospital, Sir Charles Gardiner Hospital Mental Health Service or NMHS Adult Hospital in the Home (HITH) services. The C2C team will in-reach to inpatient services to assist with discharge planning accepted to C2C for transition to community care.

Intensive Clinical Outreach Team - ICOT: ICOT is a treatment- and recovery-oriented multidisciplinary service, aimed at improving the functioning and quality of life for consumers with complex mental health needs requiring intensive intervention in the community. ICOT will facilitate development or re-engagement with meaningful life roles for consumers, informed by an explicit belief that people can and do recover from mental illness

Inpatient (See [MH Inpatient Mental Health Services Admission to Discharge Policy](#) for admission procedures)

External Community Mental Health Services – Any Mental Health Service that does not come under the governance of MHPHDS – Adult Community Mental Health

4. Key Points

- The primary objective of this procedure is to maintain consumer well-being and provide optimal care during the transfer period by meeting clinical needs.
- It is the responsibility of all workers to familiarise themselves with this procedure.
- The transferring team is responsible for organising transfer arrangements and providing continuing care where necessary until allocated to the receiving treating team or service (excluding direct transfers).
- Safety, continuing care, and good communication between all parties must be prioritised.
- This policy should be read in conjunction with the following:
 - [Mental Health Framework for State-wide Standardised Clinical Documentation – Triage to Discharge](#)



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- [Admission to Discharge Community Mental Health Services Policy](#)
 - [MH Case Management Procedure](#)
 - [MH Discharge Planning for Adult Community Consumer Procedure](#)

5. Process

The transfer process is based on the expectation that the future needs of the consumer have been discussed at the referring teams MDT and the team agree the consumer requires further treatment and support.

- Adult Community Mental Health services, for the most part, support voluntary consumers.
- To be considered for transfer a consumer must meet the criteria of:
 - having a severe and enduring mental illness and/or an acute risk to self or others the nature of which indicates the need for ongoing mental health treatment/support;
 - be willing to engage in ongoing treatment/support; or
 - currently being treated under *The Mental Health Act 2014* ([MHA 2014](#))
- The purpose for transferring a consumer to another team must be discussed in full with the consumer and/or relatives/carers by the Case Manager prior to transfer.
- Details of the handover including the name of the new Case Manager, should be documented by the receiving team in the consumer's medical record.
- Any issues or concerns specific to the transfer should be discussed Consultant to Consultant.
- If issues remain unresolved, discussion should occur with the Head of Clinical Services and the Service Coordinators.

5.1 Process for Transfer between teams

The transferring Case Manager or referring Clinician will ensure the following documentation is current and complete:

- PSOLIS Triage document
- 8-page assessment
- HoNOS
- RAMP
- Consumer self-report (K10)
- Treatment, Support and Discharge Plan
- Physical examination
- Physical Appearance Form
- Care Transfer Summary
- PSOLIS - Admission, Discharge, Transfer, and Referral form (ADTR).

If over 12 weeks:

- LSP-16

If transfer from ATT or ART

- Current 8-page Assessment

If the referring team is referring within their own service (i.e. ATT to CTT) – the current Case Manager or referring clinician sends an email, including the Care Transfer Summary or complete ISOBAR handover to all the senior staff of the receiving team, ensuring that the referring team's Clerical Officer and receiving team's Clerical Officer (or Admin Assistant) are copied in (please note the care transfer summary is on PSOLIS so may not be attached to the email).

Where possible, a verbal handover should be provided to the new Case Manager following allocation.

When transferring within our service, following the receiving teams, Clinical MDT meeting, the new Case Manager or team representative is to enter their name onto the ADTR and hand the form along with the file to the team *admin* officer (or Admin Assistant) for processing into PSOLIS. To avoid unnecessary delays, this is to be completed the same day as the team meeting.

Initial joint visits are encouraged and considered best practice – especially for high-risk consumers or consumers with multiple, complex needs.

There is no minimum length of engagement with ATT. There are occasions where a direct transfer to CTT is appropriate (see criteria below) or if it becomes evident the consumer will require more than 90 days of treatment, transfer to CTT may occur earlier.

If the referring team is referring to an external service – the current Case Manager or referring clinician should contact the receiving teams triage officer and following a verbal handover should send an email, including the Care Transfer Summary and complete ISOBAR handover to the triage officer at the receiving service, ensuring that referring team's Clerical Officer is copied in (please note the care transfer summary is on PSOLIS so may not be attached to the email).

Where possible, a verbal handover should be provided to the new Case Manager following allocation.

Initial joint visits are encouraged and considered best practice, especially for high-risk consumers or consumers with multiple, complex needs.

The team to which a consumer is transferred is primarily based on the primary residential address. Where a consumer is homeless/itinerant or of no fixed abode and the consumer requires further input, they will be transferred to the team where – at time of transfer, they are primarily residing.

On occasions where a service is being requested to take an out of area referral due to very specific consumer needs/concerns i.e. – VRO's, staff/family conflicts of interest/confidentiality, then consideration is given regarding location, accessibility, safety and the consumers best interest while accounting for the potential for undue burden on particular teams.

5.2 Direct Transfers from triage not requiring ATT

Direct transfers are referrals from an external service for consumers currently engaged in a specialist mental health service and continue to meet one of the criteria outlined below:

- In some cases, the need for CTT input will be apparent at triage. Where no urgent clinical need is identified for a Case Manager or medical review, and it is evident the consumer requires more than 90 days of treatment, transfer should be facilitated directly upon



receipt on the referral from the external service. The goal is to avoid unnecessary delays and obstacles to engagement with the team that will be providing continuity of care.

- Consumers on a CTO will be transferred directly to CTT. It is the responsibility of Triage to gather the transfer information of a consumer who has been accepted by a psychiatrist from any team within Community Adult Mental Health Service under a CTO. In the event of a consumer being transferred to a Community Adult Mental Health Service on a CTO, the referring service doctor will liaise with the receiving community team's Consultant Psychiatrist regarding the acceptance of the consumer. The accepting Consultant Psychiatrist advises Triage Officer of the notification.
- Consumers active with CTT or equivalent (i.e., regular assessment, monitoring and follow up from a specialist service) in another area.
- Consumers referred on depot or Clozapine medication will be processed by Triage and usually referred directly to CTT. In general, these consumers are likely to be suitable for direct transfer to CTT however consideration should be given regarding accommodation stability, recency of MDT input and complexity.
- Consumer has had a lengthy stay in hospital and the inpatient team consider the consumer needs more than 90 days of community treatment.

Once allocation has occurred the receiving team is to communicate with the Triage Officer to advise of the outcome.

The Triage Officer will be advised if the consumer is considered inappropriate for admission into the receiving CTT.

Three-month rule:

- If a consumer is referred to the service within 12 weeks of discharge they will be assessed by their former (discharging) team and, ideally the previous Case Manager, who will discuss the consumers current needs and discharge goals with the team to minimize the risk of relapse.
- If the consumer requires readmission:
 - PSOLIS referral document will be completed by Triage.
 - The Triage Officer will forward all referral documentation to the CTT.
 - The receiving team will discuss and allocate a Case Manager and advise the Triage officer the case Manager name for updating of PSOLIS.
 - The CTT are responsible for any crisis intervention prior to allocation or outcome.
 - The triage documentation will be completed by the receiving Case Manager or an alternative allocated team member.

6. Compliance, monitoring and evaluation

- Regular monitoring of completion of transfer suite of documentation at point of transfer by clinic staff.
- Documentation audit conducted to ensure compliance as part of the Safety Quality and Performance unit (SQPU) audit schedule.



Related internal policies, procedures and guidelines

[Mental Health Framework for State-wide Standardised Clinical Documentation – Triage to Discharge](#)

[Admission to Discharge Community Mental Health Services Policy](#)

[MH Case Management Procedure](#)

[MH Discharge Planning for Adult Community Consumer Procedure](#)

Policy Sponsor	Co-Director Community Adult Mental Health				
Policy Contact	Service Manager, Community Adult Mental Health				
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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
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04/11/2024	V1.0	Service Manager, Community Adult Mental Health	Initial endorsement

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (Insert ISD Number 3795).

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